

Guideline Title: **Hypotension**

Guideline Number: **GUID-3203-M011**

Issue date: 09/09/2014	Replaces Dept. Policy:
Revision dates: 08/11/2026	Developed by: Air Care Team
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Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** To recognize the patient with hypotension. To provide and maintain adequate oxygenation, ventilation, and tissue perfusion. To provide adequate blood pressure support and expeditious transport to the nearest appropriate medical facility.
- II. **DEFINITIONS:**
- III. **DEPARTMENT GUIDELINE:**
 - a. Establish care as defined by *GUID3203-G002 – General Patient Care*.
 - b. Establish two large bore IVs
 - c. Maintain Systolic Blood Pressure (SBP) > 90 mmHg and mean arterial pressure (MAP) > 65 mmHg
 - d. Administer a 250 ml fluid bolus (0.9 % Normal Saline or Lactated Ringers) and re-evaluate blood pressure. Repeat boluses PRN in the absence of signs and symptoms of volume overload (edema, JVD, pulmonary edema)
 - e. If actively hemorrhaging, control the bleeding and consider infusing blood products per *GUID3203-PR002 - Blood Product Administration* or *GUI3203D-PR025 – Emergency Blood Product Administration*.
 - f. If the patient remains hypotensive, administer a continuous vasopressor or inotropic infusion
 - i. **Norepinephrine 0.05-1 mcg/kg/min IV titrate q5 min to MAP > 65 mmHg** This is the *first-choice* vasopressor for sepsis, neurogenic shock, cardiogenic shock, undifferentiated shock, and bradycardia with hypotension.
 - ii. **Epinephrine 0.025-0.5 mcg/kg/min IV titrate q5 min to MAP > 65 mmHg.** This is the *first-choice* vasopressor for anaphylaxis and can be considered for refractory septic shock or shock after ROSC.
 - iii. Consider **Vasopressin 0.03 units/min IV** infusion for septic shock or hypotensive GI Bleed refractory to fluid therapy.
 - iv. Consider **Dopamine 2-20 mcg/kg/min IV titrate q5 min to effect** for bradycardia or hypotension refractory to fluid therapy.
 - v. Consider inotropic support with **Dobutamine 2-20 mcg/kg/min IV titrate q5 min to effect** for cardiogenic shock refractory to norepinephrine alone.

Guideline Title: **Hypotension**

Guideline Number: **GUID-3203-M011**

- vi. Consider **Phenylephrine 0.5-5mcg/kg/min IV titrate q5 min to MAP > 65 mmHg** for known spinal shock.
- g. If the patient is hypotensive, a Push Dose Pressor (PDP) may be used to temporize patient while setting up continuous vasopressor.
 - i. Consider **Phenylephrine 200 mcg IV push**
 - 1. Phenylephrine is contraindicated if the heart rate is < 60 bpm
 - 2. May repeat every two minutes if the SBP remains less than 90 mmHg
 - 3. PDPs provide a transient improvement in pressure meant to be a bridge to continuous vasopressor therapy. *They may only be used for documented hypotension 5 minutes before starting continuous vasopressor therapy.*
 - 4. PDPs may only be administered from approved prefilled syringes obtained from pharmacy – it is prohibited for any RN or Paramedic to mix their own PDP
- h. Consider insertion of indwelling urinary catheter with urometer at the referring facility to accurately monitor urine output if available.
- i. Consider obtaining atrial line placement if at sending facility and continue atrial blood pressure monitoring for patients being transported on antihypertensive medications

IV. **DOCUMENTATION:** EMS Charts

V. **REFERENCES:**